This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Naproxen or Mefenamic acid for Period Pain (Dysmenorrhoea)

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE CKS Guidelines for Period Pain (Primary Dysmenorrhoea)

Overview

- **Primary dysmenorrhoea** is crampy lower abdominal pain occurring before or during menstruation **without underlying pelvic pathology**.
- Most common in **adolescents and young women**, often improving with age or after childbirth.

Symptoms

- Crampy pelvic pain, often starting **a few days before** or at the onset of menstruation.

- May radiate to the **lower back or thighs**.
- Can be accompanied by **nausea**, **fatigue**, **headache**, or **diarrhoea**.

Assessment

- Diagnosis is **clinical** based on history and pattern of symptoms.
- Consider **secondary causes** (e.g. endometriosis, fibroids, pelvic inflammatory disease) if:
    - Symptoms start later in life.
    - Pain is severe, progressively worsening, or not responding to treatment.
    - Intermenstrual or postcoital bleeding, dyspareunia, or abnormal discharge is present.

Management

First-Line Treatment

1. NSAIDs (Non-Steroidal Anti-Inflammatory Drugs)

- **Ibuprofen**, **naproxen**, or **mefenamic acid** are first-line.
- Take **regularly** from onset of pain or 1–2 days before menstruation if predictable.
- Contraindications: peptic ulcer disease, renal impairment, aspirin sensitivity.

2. Hormonal Contraceptives

- Combined oral contraceptive pill (COC) can reduce menstrual flow and pain.
- Can be taken **continuously** to suppress menstruation.
- Alternatives: progestogen-only pill (POP), contraceptive injection, implant, or IUS (e.g. Mirena).

Non-Pharmacological Options

- Heat therapy (e.g. hot water bottle, heat patches).
- Regular exercise.
- Relaxation techniques, acupuncture (limited evidence).

When to Investigate or Refer

- **Suspected secondary dysmenorrhoea** (e.g. endometriosis).
- Failure of first-line treatment.
- Features suggestive of underlying pathology.

- Consider pelvic ultrasound or gynaecology referral.

Follow-Up

- Review after 3 months if treatment initiated.
- Ensure medication adherence and check for side effects.
- Consider alternative diagnoses if no improvement.

Patient Group Direction

for the supply of

Naproxen

for the treatment of

Period Pain (Dysmenorrhoea)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Naproxen is indicated for the short-term relief of primary dysmenorrhoea (period pain) in females aged 16 years and older who require treatment where paracetamol or antispasmodics are insufficient.
Inclusion criteria	- Female patients aged 16 years or older. - Presenting with symptoms of primary dysmenorrhoea. - Paracetamol or an antispasmodic has been tried for at least one cycle and was insufficient, or is unsuitable for this patient, with the answer recorded. - No contraindications to NSAID therapy. - Informed consent obtained. - Requesting treatment for period-related pain management.
Exclusion criteria	- Known or suspected pregnancy. - Breastfeeding. - History of peptic ulcer disease or gastrointestinal bleeding. - NSAID or aspirin hypersensitivity or hypersensitivity to any of the ingredients - Severe hepatic, renal, or cardiac impairment. - Concurrent use of other NSAIDs (including aspirin) or anticoagulants. - Clinically significant interacting medication - Patients who have coagulation disorders or are receiving drug therapy that interferes with haemostasis - Inflammatory bowel disease (ulcerative colitis or Crohn's disease).
Cautions	- Administer with or after food to reduce gastrointestinal irritation. - Use with caution in individuals with asthma or a history of gastrointestinal conditions. - Advise to avoid alcohol and monitor for symptoms of gastrointestinal discomfort or bleeding. - Short-term use only; reassess if pain persists or worsens. - Concomitant administration of antacids or colestyramine can delay the absorption of naproxen but does not affect its action. - Respiratory disorders: Caution is required if administered to patients suffering from, or with a previous history of, bronchial asthma since NSAIDs have been reported to precipitate bronchospasm in such patients. - Undesirable effects may be minimised by using the lowest effective dose for the shortest duration necessary to control symptoms. Patients treated with NSAIDs long-term should undergo regular medical supervision to monitor for adverse events. - The antipyretic and anti-inflammatory activities of naproxen may reduce fever and inflammation, thereby diminishing their utility as diagnostic signs. - As with other non-steroidal anti-inflammatory drugs, elevations of one

	or more liver function tests may occur. Hepatic abnormalities may be the result of hypersensitivity rather than direct toxicity. Severe hepatic reactions, including jaundice and hepatitis (some cases of hepatitis have been fatal) have been reported with this drug as with other non-steroidal anti-inflammatory drugs. Cross reactivity has been reported. - Naproxen decreases platelet aggregation and prolongs bleeding time. This effect should be kept in mind when bleeding times are determined. - Although sodium retention has not been reported in metabolic studies, it is possible that patients with questionable or compromised cardiac function may be at a greater risk when taking naproxen. - Gastrointestinal bleeding, ulceration or perforation, which can be fatal, has been reported with all NSAIDs at any time during treatment, with or without warning symptoms or a previous history of serious GI events. - Patients should report any unusual abdominal symptoms (especially GI bleeding) and discontinue the medication until reviewed. - Care should be advised in patients receiving concomitant medications which could increase the risk of ulceration or bleeding, such as oral corticosteroids, selective serotonin-reuptake inhibitors or antiplatelet agents. - When GI bleeding or ulceration occurs in patients receiving naproxen, the treatment should be withdrawn and the patient urgently referred for follow up. - NSAIDs should be given with care to patients with a history of gastrointestinal disease (ulcerative colitis, Crohn's disease) as these conditions may be exacerbated. - There have been reports of impaired renal function, renal failure, acute interstitial nephritis, haematuria proteinuria, renal papillary necrosis and occasionally nephritic syndrome associated with naproxen. - The administration of an NSAID may cause a dose dependent reduction in prostaglandin formation and precipitate renal failure. Patients at greatest risk of this reaction are those with impaired renal function, cardiac impairment, liver dysfunction, those taking diuretics, angiotensin converting enzyme inhibitors, angiotensin-II receptor antagonists and the older people. Renal function should be monitored in these patients. - As naproxen is eliminated to a large extent (95%) by urinary excretion via glomerular filtration, it should be used with great caution in patients with impaired renal function and the monitoring of serum creatinine and/or creatinine clearance is advised and patients should be adequately hydrated. Naproxen is contraindicated in patients having a baseline creatinine clearance of less than 30 ml/minute. - Haemodialysis does not decrease the plasma concentration of naproxen because of the high degree of protein binding. - Certain patients, specifically those whose renal blood flow is compromised, such as in extracellular volume depletion, cirrhosis of the liver, sodium restriction, congestive heart failure, and pre-existing renal disease, should have renal function assessed before and during

	naproxen therapy. Some elderly people in whom impaired renal function may be expected, as well as patients using diuretics, may also fall within this category. A reduction in daily dosage should be considered to avoid the possibility of excessive accumulation of naproxen metabolites in these patients. - Chronic alcoholic liver disease and probably also other forms of cirrhosis reduce the total plasma concentration of naproxen, but the plasma concentration of unbound naproxen is increased. The implication of this finding for naproxen dosing is unknown but it is prudent to use the lowest effective dose. - Hypersensitivity reactions may occur in susceptible individuals. Anaphylactic (anaphylactoid) reactions may occur both in patients with and without a history of hypersensitivity or exposure to aspirin, other non-steroidal anti-inflammatory drugs or naproxen-containing products. They may also occur in individuals with a history of angio-oedema, bronchospastic reactivity (e.g. asthma), rhinitis and nasal polyps. Anaphylactoid reactions, like anaphylaxis, may have a fatal outcome. - If steroid dosage is reduced or eliminated during therapy, the steroid dosage should be reduced slowly and the patients must be observed closely for any evidence of adverse effects, including adrenal insufficiency and exacerbation of symptoms of arthritis. - Studies have not shown changes in the eye attributable to naproxen administration. In rare cases, adverse ocular disorders including papillitis, retrobulbar optic neuritis and papilloedema, have been reported in users of NSAIDs including naproxen, although a cause-and-effect relationship cannot be established; accordingly, patients who develop visual disturbances during treatment with naproxen-containing products should have an ophthalmological examination. - Appropriate monitoring and advice are required for patients with a history of hypertension and/or mild to moderate congestive heart failure as fluid retention and oedema have been reported in association with NSAID therapy. - Clinical trial and epidemiological data suggest that use of coxibs and some NSAIDs (particularly at high doses and in long term treatment) may be associated with a small increased risk of arterial thrombotic events (for example myocardial infarction or stroke). Although data suggest that the use of naproxen (1000 mg daily) may be associated with a lower risk, some risk cannot be excluded. - Patients with uncontrolled hypertension, congestive heart failure, established ischaemic heart disease, peripheral arterial disease, and/or cerebrovascular disease should only be treated with naproxen after careful consideration. Similar consideration should be made before initiating longer-term treatment of patients with risk factors for cardiovascular events (e.g. hypertension, hyperlipidaemia, diabetes mellitus, smoking). - In patients with systemic lupus erythematosus (SLE) and mixed

	connective tissue disorders there may be an increased risk of aseptic meningitis. - Stevens-Johnson syndrome (SJS), and toxic epidermal necrolysis (TEN), and drug reaction with eosinophilia and systemic symptoms (DRESS), which can be life-threatening or fatal, have been reported post-marketing in association with naproxen treatment. If signs and symptoms suggestive of these reactions appear, naproxen should be withdrawn immediately. If the patient has developed SJS, or TEN or DRESS with the use of naproxen, treatment with naproxen must not be restarted and should be permanently discontinued. - Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens Johnson syndrome, and toxic epidermal necrolysis, have been reported very rarely in association with the use of NSAIDs. Patients appear to be at highest risk for these reactions early in the course of therapy: the onset of the reactions occurring in the majority of cases within the first month of treatment. Naproxen should be discontinued at the first appearance of skin rash, mucosal lesions, or any other sign of hypersensitivity.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Naproxen 250 mg tablets.
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration with or after food.
Dose and frequency	Initially 500 mg followed by 250 mg every 6–8 hours as needed. Maximum dose: 1250 mg on day 1, then up to 1000 mg daily.
Quantity to be administered and/or supplied	One cycle per supply: up to 14 tablets of 250 mg (500 mg then 250 mg every 6 to 8 hours, maximum 1250 mg on day 1 and 1000 mg daily thereafter, for up to 3 days, is 13 tablets). The 500 mg tablet is not supplied under this PGD because the 250 mg maintenance dose cannot be taken from it. A further supply needs a further consultation.
Maximum or minimum treatment period	Typically, up to 3 days per menstrual cycle. Undesirable effects may be minimised by using the lowest effective dose for the shortest duration necessary to control symptoms. If symptoms are severe and do not respond to initial treatment within 3-6 months or there is any doubt about the diagnosis, refer to a gynaecologist.

Adverse effects	Nausea, heartburn, gastrointestinal discomfort, dizziness, headache. Rare: gastrointestinal bleeding or hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form and route, quantity supplied, the date the period started and the number of days supplied ● the answer given on paracetamol or an antispasmodic: what was tried and for how many cycles and that it was insufficient, or why it is unsuitable ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Seek medical advice if the pain is not controlled by the treatment, is getting worse each cycle, or is accompanied by bleeding between periods or after sex, pain during sex or an abnormal discharge, or if you feel systemically unwell. If the pain has not responded over 3 to 6 cycles, see your GP so that other causes can be considered. Avoid concomitant use of two or more NSAIDs (including aspirin) as this may increase the risk of adverse effects. Local application of heat (for example, a hot water bottle or heat patch) and transcutaneous electrical nerve stimulation (TENS) may help to reduce pain. For women who do not wish to conceive, hormonal contraception is an alternative first-line treatment and has the additional advantage of providing contraception

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
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- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	**Job title**	**Registration number**	**Signature**
Valid from date		**Expiry date**	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	17th December 2025

Patient Group Direction

for the supply of

Mefenamic Acid

for the treatment of

Period Pain (Dysmenorrhoea)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Mefenamic acid is indicated for the short-term relief of primary dysmenorrhoea (period pain) in females aged 16 years and older who require treatment where paracetamol or antispasmodics are insufficient.
Inclusion criteria	- Female patients aged 16 years or older. - Presenting with symptoms of primary dysmenorrhoea. - Paracetamol or an antispasmodic has been tried for at least one cycle and was insufficient, or is unsuitable for this patient, with the answer recorded. - No contraindications to NSAID therapy. - Informed consent obtained. - Requesting treatment for period-related pain management.
Exclusion criteria	- Known or suspected pregnancy. - Breastfeeding. - History of peptic ulcer disease or gastrointestinal bleeding. - NSAID or aspirin hypersensitivity or hypersensitivity to any of the ingredients. - Severe hepatic, renal, or cardiac impairment. - Concurrent use of other NSAIDs (including aspirin) or anticoagulants. - Clinically significant interacting medication. - Inflammatory bowel disease. - Patients who have coagulation disorders or are receiving drug therapy that interferes with haemostasis.
Cautions	- Administer with or after food to reduce gastrointestinal irritation. - Use with caution in individuals with asthma or a history of gastrointestinal conditions. - Advise to avoid alcohol and monitor for symptoms of gastrointestinal discomfort or bleeding. - Short-term use only; reassess if pain persists or worsens. - Patients on prolonged therapy should be kept under regular surveillance with particular attention to liver dysfunction, rash, blood dyscrasias or development of diarrhoea. Appearance of any of these symptoms should be regarded as an indication to stop therapy immediately - Caution should be taken in patients suffering from dehydration and renal disease. - Respiratory disorders: Caution is required if administered to patients suffering from, or with a previous history of, bronchial asthma since NSAIDs have been reported to precipitate bronchospasm in such patients. - Cardiovascular and cerebrovascular effects: Appropriate monitoring and advice are required for patients with a history of hypertension and/or mild to moderate congestive heart failure as fluid retention and

	oedema have been reported in association with NSAID therapy. - Clinical trial and epidemiological data suggest that use of some NSAIDs (particularly at high doses and in long term treatment) may be associated with a small increased risk of arterial thrombotic events (for example myocardial infarction or stroke). There are insufficient data to exclude such a risk for mefenamic acid. - Patients with uncontrolled hypertension, congestive heart failure, established ischaemic heart disease, peripheral arterial disease, and/or cerebrovascular disease should only be treated with mefenamic acid after careful consideration. Similar consideration should be made before initiating longer-term treatment of patients with risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus, smoking). - As NSAIDs can interfere with platelet function, they should be used in caution in patients with intracranial haemorrhage and bleeding diathesis. - Gastrointestinal bleeding, ulceration and perforation: GI bleeding, ulceration or perforation, which can be fatal, has been reported with all NSAIDs at any time during treatment, with or without warning symptoms or a previous history of serious GI events. Smoking and alcohol use are added risk factors. - The risk of GI bleeding, ulceration or perforation is higher with increasing NSAID doses, in patients with a history of ulcer, particularly if complicated with haemorrhage or perforation, and in the elderly. - Patients with a history of GI toxicity should report any unusual abdominal symptoms (especially GI bleeding) particularly in the initial stages of treatment. - Caution should be advised in patients receiving concomitant medications which could increase the risk of gastrotoxicity or bleeding such as corticosteroids, selective serotonin reuptake inhibitors or anti-platelet agents such as aspirin. - When GI bleeding or ulceration occurs in patients receiving mefenamic acid the treatment should be withdrawn. - SLE and mixed connective tissue disease: In patients with systemic lupus erythematosus (SLE) and mixed connective tissue disorders there may be an increased risk of aseptic meningitis. - Skin reactions: Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens-Johnson syndrome, and toxic epidermal necrolysis, have been reported in association with use of NSAIDs. Patients appear to be at highest risk of these reactions early in the course of therapy, the onset of the reaction occurring in the majority of cases within the first month of treatment. mefenamic acid should be stopped at the first appearance of skin rash, mucosal lesions or any other sign of hypersensitivity. - In dysmenorrhoea and menorrhagia lack of response should alert the physician to investigate other causes.

	- Epilepsy: Caution should be exercised when treating patients suffering from epilepsy. - In patients who are known or suspected to be poor CYP2C9 metabolisers based on previous history/experience with other CYP2C9 substrates, mefenamic acid should be administered with caution as they may have abnormally high plasma levels due to reduced metabolic clearance. - Alcohol: Concomitant consumption of alcohol with mefenamic acid may increase gastrointestinal bleeding, ulceration and perforation.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Mefenamic acid 250 mg caps or 500 mg tablets.
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration with or after food.
Dose and frequency	500 mg three times a day
Quantity to be administered and/or supplied	One cycle per supply: 18 capsules of 250 mg, or 9 tablets of 500 mg (500 mg three times a day for 3 days). A further supply needs a further consultation. Do not exceed the stated dose - Mefenamic acid can cause seizures in overdose. The National Poisons Information Service considers an ingestion of 40 mg/kg or more to be potentially toxic. This means that a woman who weighs 50 kg would only need to ingest one extra dose of 500 mg of mefenamic acid in 24 hours (total of 2000 mg) to be considered to be at risk of toxicity.
Maximum or minimum treatment period	Typically, up to 3 days per menstrual cycle. Undesirable effects may be minimised by using the lowest effective dose for the shortest duration necessary to control symptoms If symptoms are severe and do not respond to initial treatment within 3-6 months or there is any doubt about the diagnosis, refer to a gynaecologist
Adverse effects	Nausea, heartburn, gastrointestinal discomfort, dizziness, headache. Rare: gastrointestinal bleeding or hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and

	inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form and route, quantity supplied, the date the period started and the number of days supplied ● the answer given on paracetamol or an antispasmodic: what was tried and for how many cycles and that it was insufficient, or why it is unsuitable ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Seek medical advice if the pain is not controlled by the treatment, is getting worse each cycle, or is accompanied by bleeding between periods or after sex, pain during sex or an abnormal discharge, or if you feel systemically unwell. If the pain has not responded over 3 to 6 cycles, see your GP so that other causes can be considered. Avoid concomitant use of two or more NSAIDs (including aspirin) as this may increase the risk of adverse effects. Local application of heat (for example, a hot water bottle or heat patch) and transcutaneous electrical nerve stimulation (TENS) may help to reduce pain. For women who do not wish to conceive, hormonal contraception is an alternative first-line treatment and has the additional advantage of providing contraception

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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	17th December 2025

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 44: both arms now carry the inclusion criterion that paracetamol or an antispasmodic has been tried for at least one cycle and was insufficient, or is unsuitable for this patient, with the answer recorded, so the indication's wording gates supply; the records row asks for that answer.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm headings corrected from administration to supply.

Naproxen quantity reconciled with the dose row: one cycle per supply, up to 14 tablets of 250 mg; the 500 mg tablet is withdrawn from this arm because the 250 mg maintenance dose cannot be taken from it. The previous 28 x 250 mg or 14 x 500 mg was about two cycles.

Inflammatory bowel disease is an exclusion in the naproxen arm as it already was for mefenamic acid; coagulation disorders and drugs interfering with haemostasis are an exclusion in the mefenamic acid arm as they already were for naproxen.

Follow-up advice replaced: the 3 to 4 weeks template line cannot apply to a 3 day course for a monthly symptom.

Records row punctuation corrected and the cycle date and number of days supplied added.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 17 December 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

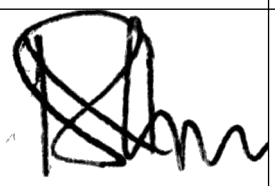
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027.

Mefenamic acid quantity: one cycle at 500 mg three times a day for 3 days is 9 doses. The previous quantity (60 x 250 mg or 30 x 500 mg) was more than three cycles of a medicine the same box describes as causing seizures in overdose with a narrow margin. Now one cycle per supply.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.